

ANALYSTLAB AFRICA

HEALTHCONNECT CLINIC EXPERIENCE LAB

PROJECT CHARTER

Improving Patient Appointment Attendance and Healthcare Support Using Data and AI

Document Control

Project	HealthConnect Clinic Experience Lab
Programme	AnalystLab Africa Experience Lab
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1. EXECUTIVE PROJECT POSITIONING

HealthConnect Clinic is experiencing challenges associated with missed patient appointments, underutilized appointment capacity and repetitive appointment-related enquiries. These challenges affect patient engagement, administrative efficiency and utilization of available appointment capacity.

HealthConnect intends to evaluate how data, machine learning and Generative AI can support improved appointment management, patient engagement and administrative support.

The project is positioned as a patient-experience and operational transformation initiative enabled by data and AI. Project delivery will integrate people, processes, data, technology and operational requirements to ensure alignment between technical outputs and intended business outcomes.

The project will be managed through the following value chain:

Patient Behaviour → Data → Insight → Decision → Intervention → Patient Response → Operational Outcome

PROJECT PRINCIPLE: Technology is an enabler of change; project success will be assessed by its contribution to measurable patient and operational outcomes.

2. BUSINESS CONTEXT

HealthConnect Clinic operates an appointment-based healthcare service in which effective scheduling, patient engagement and administrative support are essential to efficient service delivery.

The clinic is currently confronted by four connected challenges:

- Patients missing scheduled appointments
- Limited understanding of factors associated with appointment non-attendance
- Inefficient utilization of appointment slots when patients fail to attend
- Repetitive patient enquiries relating to appointments and clinic procedures

These challenges have implications for both patient experience and clinic operations. Missed appointments may result in unused capacity, while recurring enquiries may increase administrative workload.

The project will evaluate how data-driven insight, predictive intelligence, patient engagement and AI-enabled support can address these challenges.

3. PROBLEM STATEMENT

HealthConnect requires a structured, data-informed approach for understanding appointment non-attendance and improving routine patient support.

The project will address the need to:

1. Identify patterns associated with appointment attendance and non-attendance
2. Translate relevant data into actionable operational insight
3. Support timely and appropriate patient interventions
4. Improve access to routine appointment information
5. Integrate technology-enabled interventions with clinic workflows
6. Establish measures for assessing patient and operational outcomes

4. CENTRAL PROJECT QUESTION

How can HealthConnect Clinic use data and AI to reduce missed appointments and improve patient support in a way that is operationally practical, measurable and responsible?

5. PROJECT PURPOSE

The purpose of the HealthConnect project is to design and evaluate a structured, data-informed approach for improving patient appointment attendance and healthcare support.

The project will integrate analytical, technical and operational capabilities to develop solutions that can be evaluated against defined patient, operational and implementation requirements.

6. PROJECT OBJECTIVES

6.1 Primary Objective

To develop a coordinated, data-informed approach that can help HealthConnect reduce missed appointments and improve the patient support experience.

6.2 Supporting Objectives

The project will seek to:

1. Identify factors associated with appointment non-attendance
2. Determine how relevant patient and appointment data can support operational decision-making
3. Evaluate appropriate machine learning approaches for identifying appointments or patients that may benefit from proactive intervention
4. Evaluate the use of Generative AI for routine appointment-related enquiries
5. Define appropriate human oversight and escalation points for AI-supported processes
6. Determine how proposed solutions can integrate with clinic workflows
7. Establish measurable indicators for patient, operational and solution performance

7. TRANSFORMATION STATEMENT

The project seeks to support a transition from a reactive appointment-support process toward a proactive, data-informed and patient-centred operating approach.

Current State	Desired Direction
Limited visibility into appointment no-show patterns	Data-informed understanding of attendance behaviour
Patient support is largely reactive	More proactive patient engagement
Repetitive enquiries consume administrative effort	Routine enquiries supported more efficiently
Missed appointments may leave capacity unused	Earlier intervention to support improved utilisation
Technical contributions may operate independently	Integrated multidisciplinary delivery
AI may be treated as the final product	AI integrated into operational decision-making

8. PROJECT VALUE CHAIN

The project will create value through the following sequence:

PROJECT VALUE CHAIN



Technology is an enabler of change; success is measured by contribution to this chain, not by technical novelty alone.

- **Patient Behaviour** — Understand attendance patterns, engagement and support needs.
- **Data** — Collect, prepare and assess relevant information.
- **Insight** — Identify meaningful patterns, relationships and potential risk indicators.
- **Decision** — Determine whether and how intervention may be appropriate.
- **Intervention** — Deliver an appropriate reminder, support interaction or operational response.
- **Patient Response** — Assess the response to the intervention.
- **Operational Outcome** — Evaluate implications for appointment utilisation, administrative efficiency and patient experience.

9. PROJECT SUCCESS FRAMEWORK

Project success will be assessed across three dimensions.

9.1 Patient Value

- Improved appointment engagement
- More accessible appointment-related support
- Timely responses to routine enquiries
- Improved clarity around appointment procedures
- Appropriate escalation of issues requiring human assistance

9.2 Operational Value

- Better utilisation of appointment capacity

- Reduced avoidable administrative workload
- Improved visibility into appointment attendance patterns
- More informed operational decisions
- Improved coordination between patient support and clinic processes

9.3 Solution Value

- Alignment with the identified problem
- Appropriate use of relevant data
- Reliable analytical or AI outputs
- Responsible application of AI
- Usability within a realistic operational context
- Clear ownership and accountability
- Feasibility for future implementation

MEASUREMENT PRINCIPLE: Quantitative performance targets will be established after relevant baseline information, data availability and measurement methods have been validated.

10. PROJECT SCOPE

10.1 In Scope

- Analysis of appointment attendance and no-show patterns
- Review of relevant patient and appointment data
- Assessment of data quality and suitability
- Identification of factors associated with non-attendance
- Evaluation of predictive analytics and machine learning approaches
- Design of proactive patient engagement interventions
- Evaluation of Generative AI support for routine appointment-related enquiries
- Definition of human escalation and oversight requirements
- Integration of proposed solutions with clinic workflows
- Testing and evaluation of proposed approaches
- Development of implementation and monitoring considerations
- Establishment of an approach for measuring outcomes
- Coordination of multidisciplinary project workstreams

10.2 Out of Scope

The following are outside the initial project boundary unless formally approved:

- ✗ Diagnosis or treatment of medical conditions
- ✗ Autonomous clinical decision-making
- ✗ Replacement of healthcare professionals
- ✗ Development of a complete hospital information system
- ✗ Unrelated healthcare use cases
- ✗ Production deployment without appropriate validation and approval
- ✗ Uncontrolled automated communication with patients
- ✗ Use of patient information beyond the approved project purpose

11. PROJECT WORKSTREAMS

HealthConnect will be delivered through interconnected workstreams.

Workstream	Primary Responsibility
Data & Analytics	Data understanding, quality assessment, exploratory analysis and identification of attendance patterns
Predictive Intelligence	No-show prediction, model development, evaluation and interpretation
Patient Engagement	Appointment reminders, proactive engagement and intervention design
Generative AI Support	Routine enquiries, FAQ support, conversational workflows and escalation
Process & Operational Integration	Workflow mapping, operational dependencies, roles and handoffs
Testing & Validation	Functional testing, output validation, usability, risk assessment and refinement
Implementation & Adoption	Deployment considerations, stakeholder readiness, training, monitoring and continuous improvement

Project Management Integration:

Workstream Outputs → Dependencies → Integrated Decisions → Delivery

12. PROJECT MANAGEMENT RESPONSIBILITY

The Project Management function will provide the overall delivery structure for the project. Key responsibilities include:

- Establishing and maintaining project scope
- Translating objectives into manageable work packages
- Coordinating cross-workstream dependencies
- Maintaining the project schedule and milestones
- Monitoring risks, assumptions and constraints

- Supporting stakeholder alignment
- Establishing communication and reporting structures
- Tracking deliverables and project progress
- Supporting quality and readiness reviews
- Managing changes to agreed project boundaries
- Maintaining alignment between project outputs and intended business value

13. STAKEHOLDER LANDSCAPE

Stakeholder Group	Primary Interest / Role
HealthConnect Clinic Management	Strategic direction, business value, resources and project outcomes
Clinic & Administrative Staff	Workflow efficiency, usability and administrative support
Healthcare Professionals	Appropriate patient support and escalation
Patients	Accessible, timely and useful appointment support
Data & Analytics Team	Data availability, quality and analytical validity
AI/ML Team	Model development, evaluation and interpretation
Generative AI Team	Patient-support capability, safety and escalation
Project Management Team	Planning, coordination, governance, risk and delivery
Project Sponsors / Supervisors	Direction, quality, accountability and decision-making

A detailed stakeholder assessment will consider influence, interest, expectations, decision authority and engagement requirements.

14. GOVERNANCE APPROACH

HealthConnect will operate using a human-in-the-loop governance model.

Analytical and AI outputs will support decision-making rather than independently determine patient or clinical actions. Where appropriate, the decision pathway will follow:

Insight → Recommendation → Human Review → Action

Governance will focus on:

- Scope control
- Schedule and milestone control
- Risk and dependency management
- Stakeholder coordination
- Quality assurance

- Change management
- Solution readiness
- Outcome measurement

15. PROJECT ASSUMPTIONS

The project currently assumes that:

1. Relevant appointment and patient-related data will be available for analysis
2. Available data will contain sufficient information to investigate attendance patterns
3. Data can be accessed and used within the project's defined purpose
4. Specialist teams will collaborate through shared project objectives
5. Proposed AI approaches can be evaluated within the available project timeframe
6. Responsible-AI and data-protection requirements will be incorporated into solution development
7. Relevant stakeholders will provide sufficient feedback for workflow and solution validation
8. The HealthConnect operating environment will provide an adequate basis for solution design

Assumptions will be reviewed as new information becomes available.

16. PROJECT CONSTRAINTS

Potential constraints include:

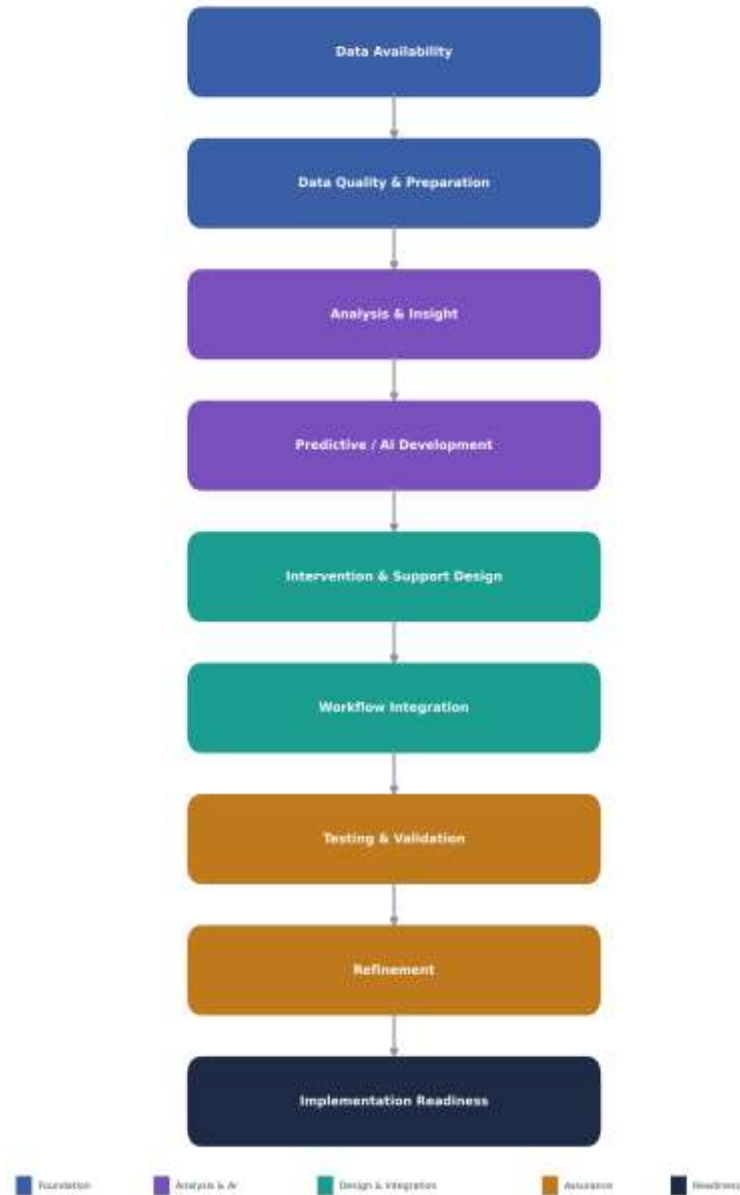
- Limited or simulated healthcare data
- Data quality and missing information
- Limited project duration and resources
- Limited access to real-world clinic workflows
- Absence of validated operational baseline metrics
- Cross-functional coordination requirements
- Technical limitations of proposed AI solutions
- Privacy and responsible-use requirements surrounding patient information
- Limitations in validating real-world patient behaviour

17. KEY PROJECT DEPENDENCIES

The project contains several significant dependencies:

KEY PROJECT DEPENDENCIES

Issues affecting upstream activities may create schedule, quality or resource implications for downstream activities



Cross-functional dependencies will also exist between data, technology, patient engagement and operational teams. Issues affecting upstream activities may create schedule, quality or resource implications for downstream activities.

18. INITIAL RISK POSITION

Risk	Potential Impact	Initial Response
Poor or incomplete data	Weak analysis and unreliable predictions	<i>Conduct early data-quality assessment</i>
Limited understanding of clinic workflows	Solution may be operationally impractical	<i>Validate workflows with stakeholders</i>
Unreliable model predictions	Incorrect prioritisation of interventions	<i>Apply appropriate model evaluation and validation</i>
Inappropriate AI-generated responses	Patient dissatisfaction, misinformation or loss of trust	<i>Establish guardrails and human escalation</i>
Cross-workstream coordination gaps	Conflicting, duplicated or incompatible outputs	<i>Establish shared milestones and coordination mechanisms</i>
Scope expansion	Schedule pressure and diluted project focus	<i>Maintain explicit scope boundaries and change control</i>
Lack of baseline metrics	Difficulty demonstrating project impact	<i>Define baseline and measurement requirements early</i>
Low stakeholder adoption	Proposed solution may not be used effectively	<i>Involve stakeholders throughout design and testing</i>
Workflow integration challenges	Technical solution may not translate into operational action	<i>Assess workflow fit before implementation</i>
Over-reliance on technical performance	Limited evidence of business value	<i>Evaluate technical and operational outcomes together</i>

19. HIGH-LEVEL PROJECT DELIVERABLES

The project is expected to produce an integrated set of outputs across its lifecycle, including:

1. Defined problem and requirements
2. Data understanding and preparation
3. Analytical findings
4. No-show prediction approach
5. Patient engagement and intervention concept
6. Generative AI support capability
7. Integrated patient-support workflow
8. Testing and validation results
9. Implementation and adoption considerations
10. Monitoring and evaluation framework
11. Final integrated solution and recommendations

The form and level of detail of each deliverable will be refined as requirements, data and solution feasibility are established.

20. HIGH-LEVEL DELIVERY ROADMAP

Phase	Focus	Outcome
1. Problem Understanding	Problem, users, data, constraints and success measures	<i>Defined project requirements</i>
2. Analysis & Solution Design	Data analysis and solution design	<i>Evidence-based solution direction</i>
3. Development	Analytical, AI and workflow components	<i>Working solution components</i>
4. Testing & Refinement	Technical, usability and workflow validation	<i>Validated and refined solution</i>
5. Implementation Readiness	Adoption, monitoring and operational considerations	<i>Implementation pathway</i>
6. Evaluation & Presentation	Outcomes, value and recommendations	<i>Integrated project conclusion</i>

21. PROJECT DECISION-MAKING PRINCIPLES

Project decisions will be guided by six principles.

1. Patient Value Before Technical Novelty

Technology should address a defined patient or operational need.

2. Evidence Before Assumption

Available evidence should guide decisions, while assumptions should be explicitly documented where evidence is unavailable.

3. Integration Before Isolation

Workstreams should be designed around their dependencies rather than developed as disconnected outputs.

4. Human Oversight Where Appropriate

AI should augment responsible decision-making rather than independently determine patient or clinical actions.

5. Feasibility Alongside Ambition

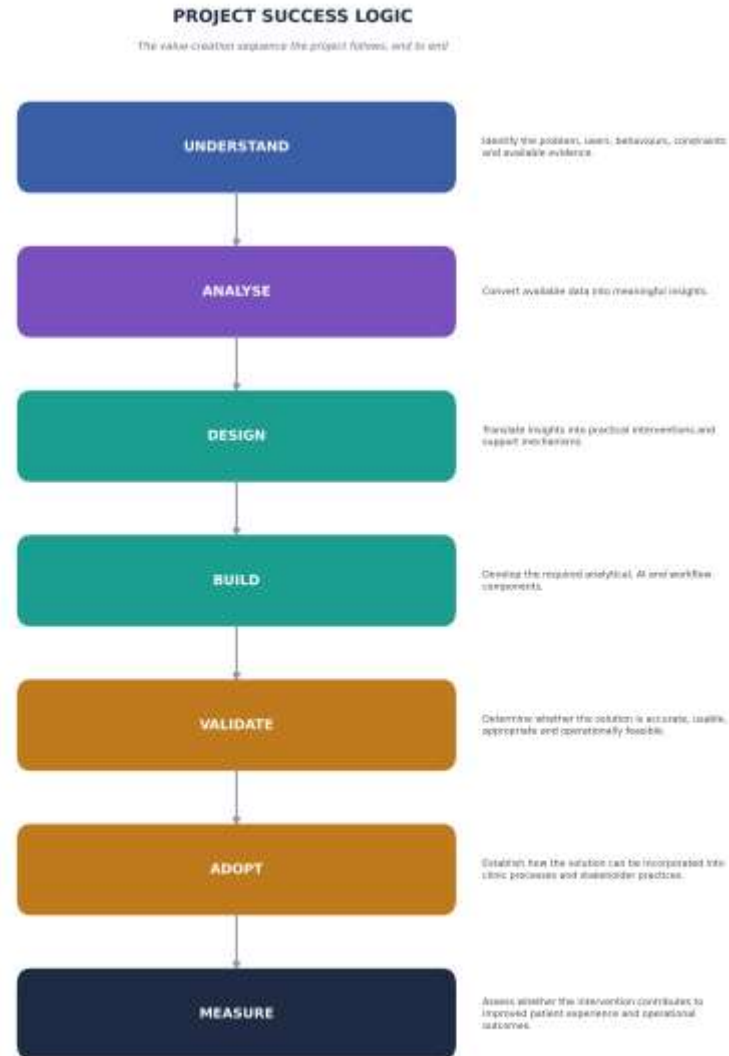
Potential impact should be balanced against technical, operational and organisational feasibility.

6. Measurable Value

Project performance should be evaluated against defined measures of patient, operational and solution value.

22. PROJECT SUCCESS LOGIC

The project will progress through the following value-creation sequence:



23. PROJECT BOUNDARY AT INITIAT

ION

At initiation, HealthConnect establishes what the project exists — and does not exist — to do.

THE PROJECT EXISTS TO:

- ✓ Improve understanding of appointment non-attendance
- ✓ Explore interventions that can support better appointment attendance
- ✓ Improve the efficiency and accessibility of routine patient support
- ✓ Investigate appropriate applications of data and AI
- ✓ Integrate technical solutions with practical clinic workflows
- ✓ Establish a basis for measuring patient and operational value

THE PROJECT DOES NOT EXIST TO:

- ✗ Replace clinical judgement
- ✗ Automate healthcare decisions without appropriate oversight
- ✗ Build technology without a defined operational purpose
- ✗ Address unrelated healthcare challenges
- ✗ Treat model performance as a substitute for real-world impact

These boundaries will provide the basis for maintaining project focus as requirements evolve.

24. PROJECT GOVERNANCE STRUCTURE

The project will operate through coordinated ownership across its major workstreams. The Project Management function will provide the overarching delivery structure, while specialist teams will retain responsibility for the quality and validity of their respective outputs.

Governance Relationship:

Project Direction → Workstream Ownership → Cross-Workstream Dependencies → Integrated Decisions → Validation & Assurance → Implementation Readiness

Key governance decisions will address:

- Scope
- Schedule
- Risks
- Dependencies
- Quality
- Stakeholder alignment
- Changes
- Solution readiness
- Expected value

25. PROJECT SUCCESS CRITERIA

HealthConnect will be considered successful where the project demonstrates a measurable relationship between the identified problem, proposed intervention and intended outcome. Success will be assessed through:

Patient

Improved ability of patients to engage with and receive support from the clinic.

Operations

Improved utilisation of appointment capacity and administrative resources.

Solution

A reliable, usable, responsible and feasible data- and AI-enabled approach.

Quantitative targets will be established once appropriate baseline information and measurement methods have been confirmed.

26. PROJECT CHARTER CLOSURE

This charter establishes the strategic direction, objectives, boundaries, governance principles and value framework for the HealthConnect Clinic Experience Lab.

HealthConnect will be managed as an integrated patient-experience and operational transformation project, with data and AI serving as enabling capabilities. The project will maintain alignment between:

Business Need → Patient Need → Data & Insight → Solution → Operational Adoption → Measurable Value

Central Project Question:

How can HealthConnect Clinic use data and AI to reduce missed appointments and improve patient support in a way that is operationally practical, measurable and responsible?

Document Status

Document	HealthConnect Project Charter
Version	1.0
Project Phase	Initiation
Status	Approved for Project Initiation
Project Management Owner	Project Management Function
Review Authority	Project Sponsor / Project Leadership
Next Review	Following completion of initial project planning

End of Project Charter